

Patient Placement and Bed Management Procedure — Medical Center West

Tallowbrook Health | Capacity Management, Medical Center West | 2025-09-15 | v4.2

Procedure CM-104, revision 4.2. Applies to all inpatient placements at Tallowbrook Medical Center West, including admissions from the emergency department, direct admissions, post-procedure admissions and internal transfers. Revision 4.2 updates the escalation contacts and restates the boarding measurement definition; the workflow is unchanged from revision 4.1.

Scope

This procedure covers the 192 staffed inpatient beds at Medical Center West: ICU (20), Progressive Care (24), Telemetry (28), 4 West (32), 5 West (30), 6 West Surgical/Orthopedic (28), Mother-Baby (20) and NICU (10).

Labor and delivery triage, the NICU admission process and transfers to outside facilities follow their own procedures. They are referenced here only where they affect bed assignment.

Roles

- House supervisor: assigns beds 24 hours a day using the EHR bed board and approves placements outside a patient's preferred unit.
- Capacity management manager: leads the daily bed huddle, owns escalation during business hours and reviews exceptions weekly.
- Unit charge nurse: updates bed status, confirms expected discharges and accepts assigned patients.
- Case management: identifies expected discharges and barriers by 9:00 a.m.
- Environmental services lead: cleans vacated beds in priority order set by the house supervisor.
- Transport: moves patients after a bed is assigned and ready.

Bed request to assignment

1. The admitting provider enters the admission order in the EHR; the order creates the bed request.

- 2. The house supervisor reviews level of care and isolation needs and assigns a bed on the EHR bed board.
- 3. The receiving charge nurse accepts or raises a concern within 15 minutes.
- 4. When the bed is marked clean and ready, the ED nurse gives report and transport is requested.
- 5. The patient's arrival on the unit is recorded in the EHR, which closes the request.

Daily bed huddle

The bed huddle is held at 9:30 a.m. every day in the capacity office, with a phone bridge for unit charge nurses.

Attendees are the capacity management manager (lead), unit charge nurses, case management, the environmental services lead and transport.

Each unit reports open beds, expected discharges, pending admissions and staffing concerns. The huddle agrees on the order of discharges and bed cleaning.

When house occupancy is above 85%, a second check-in is held at 2:30 p.m.

Escalation

A bed request that is not assigned within 60 minutes is escalated by the house supervisor to the capacity management manager, or after hours to the administrator on call.

- Step 1: review expected discharges and request early discharge support from case management.
- Step 2: consider placement on an alternate unit with appropriate nursing competency.
- Step 3: open overflow capacity approved by the chief nursing officer.
- Step 4: the administrator on call decides on diversion status with the ED medical director.

Boarding measurement

ED boarding time runs from the inpatient admission order to the time the patient physically leaves the ED for an inpatient bed.

Boarding hours are attributed to the destination unit, not the ED, and are reported daily by Decision Support.

Reports before February 2024 started the clock at the bed request rather than the admission order, so older boarding figures are not directly comparable.

Exceptions and special populations

Patients requiring isolation are placed only in rooms marked for isolation on the bed board.

Pediatric patients admitted through the ED are placed according to the pediatric admission criteria in the ED procedure manual.

Patients awaiting transfer to a behavioral health hospital remain under ED care unless a medical admission is ordered.

Records

Bed requests, assignments and transfer times are recorded in the EHR ADT and bed management module and loaded nightly to the data warehouse. Huddle notes are kept by the capacity office for 90 days.